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GUIDE 05 · THE FLORA COLLECTION

The PMS Elimination Guide

End the cycle of monthly suffering — for good.

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You Don't Have to Live Like This Every Month

PMS is not a life sentence. For decades, women have been told that bloating, cramps, mood swings, and fatigue are simply the price of having a cycle. That is not true. These symptoms are signals — your body communicating that something is out of balance. This guide gives you the tools to listen, respond, and transform your luteal phase from something you dread into something you navigate with ease.

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PMS affects up to 90% of menstruating women, yet research shows that targeted nutrition, supplementation, and lifestyle changes can reduce symptoms by over 70% within two cycles.

HOW TO USE THIS GUIDE

1. Start with Section 01 to understand the hormone drivers behind your specific symptoms.
2. Complete the PMS Profile (page 5) to identify your dominant pattern.
3. Follow the 4-week protocol matched to your profile in Section 03.
4. Use the daily tracker (Section 05) to monitor progress across two full cycles.

KEY TAKEAWAY

Most women see measurable improvement within 4–6 weeks of consistent protocol implementation. The key is addressing root causes — not just masking symptoms month to month.

Why PMS Happens

PMS symptoms occur during the luteal phase — days 15 to 28 of a typical 28-day cycle. During this phase, progesterone rises sharply after ovulation and then drops before menstruation begins. When this hormonal shift is smooth, the transition is barely noticeable. When there is underlying imbalance, the crash triggers a cascade of physical and emotional symptoms.

THE SCIENCE · LUTEAL PHASE DYSPHORIA

Research from Harvard Medical School identifies three primary hormonal mechanisms behind PMS: (1) exaggerated sensitivity to normal progesterone fluctuations affecting GABA receptors in the brain; (2) relative estrogen dominance creating a high estrogen-to-progesterone ratio; and (3) serotonin depletion triggered by falling estrogen levels in the late luteal phase. Each mechanism responds to different interventions — which is why a one-size-fits-all approach rarely works.

THE THREE DRIVERS

ESTROGEN DOMINANCE

Excess estrogen relative to progesterone amplifies bloating, breast tenderness, mood swings, and heavy bleeding. Driven by poor liver detox, gut dysbiosis, and xenoestrogen exposure.

PROGESTERONE DEFICIENCY

Low progesterone causes anxiety, sleep disruption, spotting before period, and heightened PMS severity. Often rooted in chronic stress, under-eating, or luteal phase defect.

SEROTONIN IMBALANCE

Falling estrogen in the late luteal phase triggers serotonin drops, causing irritability, food cravings, depression, and poor impulse control in the days before your period.

The 28-Day Hormonal Map

Understanding when each hormone peaks and falls gives you predictive power over your symptoms. The luteal phase (days 15–28) is where PMS lives — but what happens in the follicular and ovulatory phases directly shapes how severe your luteal phase will be.

Days 1–5
MENSTRUAL

MENSTRUAL

Estrogen & progesterone at lowest. Rest, iron-rich foods.

Days 6–13
FOLLICULAR

FOLLICULAR

Estrogen rising. Energy builds. Foundation for good ovulation.

Days 14–16
OVULATORY

OVULATORY

LH & estrogen peak. Window of high energy & confidence.

Days 17–28
LUTEAL

LUTEAL

Progesterone rises then falls. PMS window. Needs most support.

THE SCIENCE · LUTEAL PHASE LENGTH

The luteal phase should last 12–16 days. A luteal phase shorter than 10 days (luteal phase defect) is a major driver of PMS and fertility challenges. This can be identified via basal body temperature charting and is often correctable through targeted progesterone support — Vitex agnus-castus being the most evidence-backed natural option.

Identify Your Pattern

PMS is not one condition — it is a cluster of patterns. Research identifies four primary PMS types, each driven by different hormonal imbalances. Identifying yours is the first step to targeted, effective treatment.

TYPE A · ANXIETY

- Irritability and mood swings
- Tension and inability to relax
- Nervous energy or restlessness
- Arguments or emotional outbursts

Driver: Estrogen excess + low progesterone affecting GABA receptors

TYPE C · CRAVINGS

- Sugar and carbohydrate cravings
- Headaches or migraines
- Fatigue and energy crashes
- Heart palpitations

Driver: Serotonin deficit + blood sugar dysregulation

TYPE D · DEPRESSION

- Low mood and tearfulness
- Memory and concentration issues
- Withdrawal and isolation
- Feelings of hopelessness

Driver: High progesterone relative to estrogen + neurotransmitter imbalance

TYPE H · HYPERHYDRATION

- Abdominal bloating and swelling
- Breast tenderness and engorgement
- Weight gain of 2–5 lbs
- Skin breakouts

Driver: Estrogen dominance causing aldosterone imbalance and fluid retention

Eat to End PMS

Food is your most powerful tool for hormonal balance. The right nutrients directly support estrogen metabolism, progesterone production, and serotonin synthesis. The wrong foods amplify every PMS symptom. This is not about restriction — it is about strategic nourishment across your cycle.

ANTI-PMS POWER FOODS

Balances estrogen via lignans; seed cycling support

Flaxseeds

Magnesium + folate for mood and cramping

Leafy Greens

Zinc supports progesterone production post-ovulation

Pumpkin Seeds

Omega-3s reduce prostaglandins, cramps, and

Wild Salmon

B6 + fiber for estrogen clearance and serotonin

Lentils

Curcumin inhibits inflammatory COX-2 pathway

Turmeric

FOODS TO AVOID (LUTEAL)

- Alcohol — depletes B vitamins
- Caffeine — worsens breast tenderness
- Refined sugar — crashes serotonin
- Excess salt — worsens bloating
- Dairy — increases prostaglandins

FOODS TO PRIORITIZE

- Complex carbs — boost serotonin
- Cruciferous veg — clear estrogen
- Dark chocolate 85%+ — magnesium
- Ginger tea — anti-cramping
- Chickpeas — B6 + zinc

Evidence-Based Supplementation

These supplements have the strongest clinical evidence for PMS relief. Start with the foundational tier for 8 weeks before adding targeted support. Always consult a healthcare provider before starting new supplements, especially if on medications.

TIER 1 FOUNDATION

- **Magnesium Glycinate**
300–400 mg/day *Most important PMS mineral — calms nervous system, reduces cramps*
- **Vitamin B6**
50–100 mg/day *Serotonin synthesis co-factor; reduces mood symptoms by up to 70%*
- **Evening Primrose Oil**
1000–3000 mg/day *GLA reduces breast tenderness and inflammatory prostaglandins*

TIER 2 TARGETED

- **Vitex Agnus-Castus**
400 mg/day *Supports progesterone; effective for Type A & luteal phase defect*
- **Calcium D-Glucarate**
500 mg/day *Enhances estrogen clearance via liver Phase II detoxification*
- **5-HTP**
50–100 mg evening *Serotonin precursor; use for Type C/D — do NOT combine with SSRIs*

□ SUPPLEMENT CAUTIONS

- 5-HTP is contraindicated with antidepressants — serotonin syndrome risk
- Vitex can affect oral contraceptive efficacy — discuss with your GP
- B6 >200 mg/day long-term may cause peripheral neuropathy
- Always cycle off supplements for 1 week per month

The Non-Negotiables

Nutrition and supplements work best on a foundation of lifestyle support. Cortisol — the stress hormone — directly suppresses progesterone and worsens every PMS symptom. Managing your nervous system is not optional; it is the bedrock of hormone health.

SLEEP

7–9 hours; prioritize consistency over duration. Melatonin and progesterone share a precursor — poor sleep directly tanks progesterone in the luteal phase.

EXERCISE TIMING

High-intensity in follicular phase; shift to yoga, walking, and swimming in the luteal phase. Intense HIIT in the luteal phase spikes cortisol and worsens PMS.

STRESS MANAGEMENT

10–20 minutes of breathwork or meditation daily reduces cortisol by measurable amounts. Box breathing (4-4-4-4) activates the parasympathetic system within 90 seconds.

ALCOHOL REDUCTION

Alcohol impairs liver estrogen clearance and depletes magnesium and B vitamins. Even 2–3 drinks per week measurably worsens PMS severity — especially in the luteal phase.

CAFFEINE TIMING

Cut caffeine after 2pm in the luteal phase. Caffeine raises cortisol and worsens breast tenderness by increasing methylxanthines in breast tissue.

Your First Month Plan

This protocol is designed for a 28-day cycle. Adapt the phase windows to your own cycle length by tracking your cycle start and ovulation date. Each week has a clear focus and targeted actions that compound across the month.

WEEK 1

Days 1–7

Menstrual Recovery

- Focus on iron-rich foods: lentils, leafy greens, pumpkin seeds
- Begin magnesium glycinate 300 mg nightly
- Gentle movement only — walking, restorative yoga
- Liver support: reduce alcohol, add cruciferous vegetables
- Begin cycle tracking in your journal or app

WEEK 2

Days 8–14

Follicular Foundation

- Add Vitamin B6 50 mg with breakfast
- Increase protein intake to support upcoming ovulation
- Higher-intensity exercise welcome — energy is high
- Add flaxseeds 2 tbsp daily for lignan estrogen balancing
- Focus on sleep consistency — 7-9 hours same bedtime

WEEK 3

Days 15–21

Ovulatory Optimization

- Shift to zinc-rich foods: pumpkin seeds, chickpeas, cashews
- Add Evening Primrose Oil 1000 mg if breast tenderness is common
- Note ovulation signs: temp rise, CM changes, LH test
- Begin winding down exercise intensity for luteal transition
- Reduce caffeine to mornings only

WEEK 4

Days 22–28

Luteal Phase Support

- Full supplement stack in effect — all Tier 1 supplements daily
- Prioritize complex carbs for serotonin: sweet potato, oats, lentils
- Daily 10-minute breathwork or meditation — non-negotiable
- Reduce alcohol and refined sugar completely this week
- Track all symptoms in daily log for cycle comparison

Work With Your Cycle

Cycle syncing means strategically adjusting your nutrition, exercise, and lifestyle to support each phase of your cycle. For PMS specifically, the luteal phase requires a significant shift from the energized follicular and ovulatory phases. Women who cycle sync report 60–80% reduction in PMS severity within three cycles.

60–80%

PMS reduction with cycle syncing

3 cycles

Average time to see full benefit

Day 22

When luteal support begins

2× impact

Synced vs. non-synced protocol

LUTEAL PHASE SURVIVAL TOOLKIT

- **Magnesium bath soaks**
Add 2 cups Epsom salts to evening bath — transdermal magnesium absorption for muscle relaxation and stress relief
- **Heat therapy for cramps**
Heat pad reduces uterine cramping by relaxing smooth muscle and increasing blood flow — as effective as low-dose ibuprofen in studies
- **Anti-inflammatory teas**
Ginger, turmeric, and chamomile teas reduce prostaglandin activity and calm the nervous system in the late luteal phase
- **Seed cycling**
Flax + pumpkin seeds (days 1–14) shifted to sesame + sunflower seeds (days 15–28) provides phase-appropriate phytoestrogen and zinc support
- **Castor oil packs**
Applied to lower abdomen 3× per week, castor oil packs reduce inflammation, support lymphatic drainage, and ease uterine congestion

Track Your Progress

Data transforms guesswork into insight. Tracking your symptoms across two full cycles allows you to see exactly which interventions are working, which symptoms are improving, and what your unique pattern looks like. Use this daily log starting on cycle day 1.

HOW TO USE THE TRACKER

1. Rate each symptom 0–3: 0 = absent, 1 = mild, 2 = moderate, 3 = severe
2. Track every day — not just on symptomatic days. Baseline data is essential.
3. Note energy (1–5), mood (1–5), and sleep quality (1–5) as additional metrics.
4. After two cycles, compare cycle 1 total scores to cycle 2 to measure improvement.
5. Bring your tracker to any healthcare appointments as objective symptom evidence.

SYMPTOM CATEGORIES TO TRACK

PHYSICAL

- Bloating / water retention
- Breast tenderness
- Cramping / pelvic pain
- Headache / migraine
- Back pain
- Fatigue / low energy
- Acne / skin changes

EMOTIONAL

- Irritability / anger
- Anxiety / tension
- Sadness / low mood
- Mood swings
- Tearfulness
- Low motivation
- Social withdrawal

COGNITIVE

- Brain fog / poor focus
- Memory lapses
- Decision fatigue
- Overwhelm
- Poor sleep quality

BEHAVIORAL

- Sugar / carb cravings
- Appetite increase
- Alcohol cravings
- Reduced libido
- Sleep changes

PMS vs. PMDD

Premenstrual Dysphoric Disorder (PMDD) is a severe form of PMS affecting 3–8% of menstruating women. It involves disabling mood symptoms in the luteal phase that significantly impair daily functioning. PMDD requires medical diagnosis and often a combination of targeted treatment beyond what lifestyle alone can address.

PMS — SELF-MANAGEABLE

- Symptoms uncomfortable but manageable
- Mood changes present but not disabling
- Can maintain work and relationships
- Symptoms clear within 2 days of period
- Lifestyle changes bring clear relief

PMDD — SEEK MEDICAL HELP

- Inability to function at work or home
- Suicidal thoughts or self-harm urges
- Rage episodes affecting relationships
- Symptoms lasting >2 weeks per cycle
- No relief from lifestyle interventions

□ SEEK IMMEDIATE HELP IF YOU EXPERIENCE

- Thoughts of self-harm or suicide in the luteal phase
- Inability to care for yourself or dependants during PMS window
- Symptoms that don't resolve after your period begins
- PMS symptoms that have significantly worsened over recent months

THE SCIENCE · PMDD TREATMENT OPTIONS

Evidence-based PMDD treatments include SSRIs (particularly effective when taken only in the luteal phase — 'luteal-phase dosing'), combined oral contraceptives with specific progestins, GnRH agonists for severe cases, and CBT-based therapies targeting the cycle-linked thought patterns. The IAPMD (International Association for PMDD) offers a global provider directory at iapmd.org.

Your PMS Action Plan

DAILY NON-NEGOTIABLES

- Magnesium glycinate 300–400 mg (every night — not just when symptomatic)
- Vitamin B6 50 mg with breakfast
- 8 glasses water — hydration directly impacts bloating and mood
- 10 minutes breathwork or meditation
- 7–9 hours sleep with consistent bed and wake times
- 2+ cups leafy greens for magnesium and folate

LUTEAL PHASE ADDITIONS (DAYS 15–28)

- Evening Primrose Oil 1000–3000 mg for breast tenderness
- Increase complex carbohydrates — sweet potato, oats, brown rice
- Eliminate alcohol and minimize caffeine
- Add castor oil pack or Epsom salt bath 3× per week
- Shift exercise to yoga, walking, swimming
- Track all symptoms daily using the log on page 15

KEY TAKEAWAY

The single most impactful change you can make: take magnesium glycinate every night starting today. Most women experience measurable improvement in sleep, mood, and cramping within 2–3 weeks — and it compounds with every additional protocol layer.

EXPECTED RECOVERY TIMELINE

Week 1–2	Magnesium + B6 foundations <i>Sleep improves; night-time tension reduces</i>
Week 3–4	First full luteal with protocol <i>20–30% symptom reduction expected</i>
Month 2	Compounding effect <i>50–60% reduction; patterns become visible</i>
Month 3+	Full optimization <i>70–80% reduction; PMS largely manageable</i>

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01 · The 28-Day Hormone Reset

02 · The Cycle Syncing Meal Bible

03 · The Hormone Type Guide

04 · The Symptom Decoder

05 · The PMS Elimination Guide

06 · The Cortisol Reset

07 · The Cycle Fitness Guide

08 · The 90-Day Rebalancing Roadmap

For educational purposes only. Not medical advice.